

SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

Session: Revision Mock Set 06	Duration: 3 hours
Specialty: Psychotherapy	Coefficient: 04
Eliminatory mark: < 8/20	Training reference: VTI-AEDA examination style

COMPREHENSIVE VOCATIONAL PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 06

Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.

How to use this marking guide

For revision, first attempt the paper without notes. Then compare not only the answer letter but the clinical cue that makes the answer correct. For legal/ethical items, the guide follows the concepts emphasized by the supplied course/examination materials; in real practice, applicable Cameroon law, institutional policy and the current professional code must also be checked.

PART ONE: PSYCHOPATHOLOGY - ANSWERS

1. During assessment, Martha reports marked fear of a specific object or situation that triggers immediate anxiety and avoidance out of proportion to the actual danger. Which of the following is the BEST match?

- A. Opioid intoxication
- B. Obsessive-compulsive personality disorder (OCPD)
- C. Specific phobia**
- D. Dependent personality disorder

Answer: C - Specific phobia. A specific phobia is circumscribed: the fear is linked to a particular object or situation and typically produces avoidance.

Memory aid: SPECIFIC = one SPECIFIC trigger.

2. A therapist notes that Patricia has marked fear of social or performance situations because of possible scrutiny, embarrassment or negative evaluation. Which condition should be considered first from the options below?

- A. Social anxiety disorder**
- B. Schizophrenia
- C. Substance use disorder
- D. Dependent personality disorder

Answer: A - Social anxiety disorder. Social anxiety centers on being observed, judged or embarrassed in social/performance settings.

Memory aid: SOCIAL = Scrutiny Of Client In Activities with Lots of people.

3. During assessment, Wendy reports fear and avoidance of situations where escape may be difficult or help unavailable, such as crowds, public transport or being outside alone. Which of the following is the BEST match?

- A. Schizophrenia
- B. Obsessive-compulsive personality disorder (OCPD)
- C. Agoraphobia**
- D. Binge-eating disorder

Answer: C - Agoraphobia. Agoraphobia concerns situations in which escape or access to help feels difficult; it may occur with or without panic disorder.

Memory aid: AGORA = Afraid to Go Out/Risk being Away from help.

4. A therapist notes that Victor has intrusive unwanted thoughts or urges and/or repetitive behaviors or mental acts performed to reduce anxiety. Which condition should be considered first from the options below?

- A. Major depressive disorder
- B. Schizophrenia

C. Obsessive-compulsive disorder (OCD)

D. Autism spectrum disorder

Answer: C - Obsessive-compulsive disorder (OCD). OCD involves obsessions (intrusive thoughts/urges/images) and compulsions (repetitive acts or mental rituals).

Memory aid: O = Obsession is a thought; C = Compulsion is a Conduct.

5. During assessment, Joel reports after trauma, recurrent intrusions or nightmares, avoidance, negative changes in mood/cognition and heightened arousal persisting beyond one month. Which of the following is the BEST match?

A. Generalized anxiety disorder (GAD)

B. Post-traumatic stress disorder (PTSD)

C. Bipolar disorder

D. Substance use disorder

Answer: B - Post-traumatic stress disorder (PTSD). PTSD follows exposure to trauma and is organized around intrusion, avoidance, negative cognitive/mood changes and arousal/reactivity.

Memory aid: IANA: Intrusion, Avoidance, Negative changes, Arousal.

6. A therapist notes that Zachary has at least two weeks of depressed mood and/or loss of interest with associated sleep, appetite, energy, concentration, guilt, psychomotor or suicidal symptoms. Which condition should be considered first from the options below?

A. Major depressive disorder

B. Agoraphobia

C. Obsessive-compulsive personality disorder (OCPD)

D. Specific phobia

Answer: A - Major depressive disorder. Major depression is a sustained depressive syndrome, not simply transient sadness. Functional impairment and risk, especially suicidality, must be assessed.

Memory aid: SIGECAPS: Sleep, Interest, Guilt, Energy, Concentration, Appetite, Psychomotor, Suicide.

7. Victor, a 32-year-old laboratory technician, presents with abnormally elevated or irritable mood with increased energy, reduced need for sleep, pressured speech, grandiosity, racing thoughts and risky activity. Which diagnosis or clinical concept best fits this presentation?

A. Manic episode

B. Specific phobia

C. Obsessive-compulsive personality disorder (OCPD)

D. Major depressive disorder

Answer: A - Manic episode. A manic episode combines abnormal mood elevation/irritability with increased energy and characteristic activation symptoms.

Memory aid: DIG FAST: Distractible, Indiscretion, Grandiosity, Flight of ideas, Activity, Sleep less, Talkative.

8. During assessment, Rose reports a history of a manic episode, often alternating with periods of depression. Which of the following is the BEST match?

A. Bipolar disorder

B. Dependent personality disorder

C. Major depressive disorder

D. Specific phobia

Answer: A - Bipolar disorder. The defining feature of bipolar I disorder is a manic episode; depressive episodes are common but not required for that diagnosis.

Memory aid: BI-polar = mood can move toward two poles; mania is the key flag.

9. During assessment, Amina reports persistent psychotic illness with delusions, hallucinations and/or disorganized speech or behavior, often with social or occupational dysfunction. Which of the following is the BEST match?

A. Substance use disorder

B. Dependent personality disorder

C. Schizophrenia

D. Social anxiety disorder

Answer: C - Schizophrenia. Schizophrenia is a chronic psychotic disorder; diagnosis depends on characteristic symptoms, duration and functional impact, not on a laboratory test alone.

Memory aid: 3D psychosis: Delusions, perceptual Disturbance, Disorganization.

10. Kemi, a 19-year-old teacher, presents with sudden onset of delusions, hallucinations or disorganized speech/behavior lasting at least a day but less than a month, with return toward baseline. Which diagnosis or clinical concept best fits this presentation?

A. Panic disorder

B. Anorexia nervosa, restricting type

C. Brief psychotic disorder

D. Agoraphobia

Answer: C - Brief psychotic disorder. The very short duration and return toward premorbid functioning distinguish brief psychotic disorder from longer psychotic disorders.

Memory aid: BRIEF = psychosis Below one month.

SECTION B: TRUE/FALSE - EXPLANATIONS

1. TRUE. Reduced need for sleep, grandiosity and pressured speech are compatible with mania.

Those are classic manic activation features.

2. FALSE. Bulimia nervosa necessarily requires markedly low body weight.

Markedly low weight is central to anorexia; bulimia can occur at many body weights.

3. TRUE. Binge-eating disorder is defined by recurrent binge eating without regular compensatory behaviors.

Regular purging or other compensation would point more toward bulimia if other criteria are met.

4. FALSE. OCPD and OCD are identical diagnoses.

OCPD is a personality pattern of perfectionism/control; OCD centers on obsessions and compulsions.

5. TRUE. Borderline personality disorder may include abandonment fears and recurrent self-injury.

Both are characteristic features of borderline personality disorder.

SECTION C: MODEL SHORT ANSWERS

1. Differentiate OCD from obsessive-compulsive personality disorder. (2mks)

- OCD involves obsessions and/or compulsions causing distress/time burden.

- OCPD is a pervasive pattern of perfectionism, orderliness and control; obsessions/compulsions are not required.

Memory aid: OCD = obsessions/compulsions; OCPD = order/control/personality.

2. Give two signs that suggest psychosis. (2mks)

- Examples: delusions, hallucinations, markedly disorganized speech/behavior, impaired reality testing.

Memory aid: 3D: Delusions, perceptual Disturbance, Disorganization.

3. Why must suicide risk be assessed in a severely depressed client? (2mks)

- Depression may involve hopelessness and suicidal ideation; assessment identifies immediacy of risk and need for protective intervention/referral.

Memory aid: Ask directly about thoughts, plan, intent, means and protective factors.

4. State two differences between GAD and a specific phobia. (2mks)

- GAD: diffuse worry across multiple domains; specific phobia: circumscribed fear of a particular object/situation.

- GAD is persistent generalized worry; phobia is triggered by the feared stimulus and avoidance.

Memory aid: General vs specific.

5. What does DSM refer to in psychopathology? (2mks)

- Diagnostic and Statistical Manual of Mental Disorders.

- A classification system used to describe diagnostic criteria; it supports but does not replace clinical assessment.

Memory aid: DSM = Diagnostic + Statistical + Manual.

PART TWO: THEORIES/TECHNIQUES - ANSWERS

1. Which psychotherapy concept is most accurately described by the following: learning by association between stimuli so that a previously neutral cue comes to elicit a response?

- A. Social skills training
- B. Classical conditioning**
- C. Retroflexion
- D. Adlerian therapy

Answer: B - Classical conditioning. Classical conditioning explains learned associations such as fear conditioning and underlies some exposure-based principles.

Memory aid: *Classical = cue-to-cue association.*

2. Which psychotherapy concept is most accurately described by the following: behavior is shaped by its consequences, including reinforcement and punishment?

- A. Progressive muscle relaxation
- B. Cognitive behavioral therapy (CBT)
- C. Systematic desensitization
- D. Operant conditioning**

Answer: D - Operant conditioning. Operant conditioning focuses on consequences that increase or decrease behavior.

Memory aid: *Operant = behavior OPERATES, consequence follows.*

3. A therapist working with Victor uses an intervention characterized by a behavior increases because it removes or prevents an aversive stimulus. Which approach or technique is being used?

- A. Existential therapy
- B. Gestalt therapy
- C. Negative reinforcement**
- D. Free association

Answer: C - Negative reinforcement. Negative reinforcement strengthens behavior; negative means something unpleasant is removed, not that behavior is punished.

Memory aid: *Reinforcement always raises behavior; negative removes something.*

4. Which psychotherapy concept is most accurately described by the following: a previously reinforced behavior decreases when the reinforcement maintaining it is consistently withheld?

- A. Extinction**
- B. Rational emotive imagery
- C. Psychoanalytic therapy
- D. Empty-chair technique

Answer: A - Extinction. Extinction occurs when reinforcement is no longer provided for a behavior.

Memory aid: *Extinction = extinguish the payoff.*

5. Which psychotherapy concept is most accurately described by the following: learning behavior by observing what happens to another person or by imitating a model?

- A. Classical conditioning
- B. Negative reinforcement
- C. Social modeling**
- D. Progressive muscle relaxation

Answer: C - Social modeling. Modeling is observational learning: the client learns from another person's behavior and its consequences.

Memory aid: *MODEL = learn by watching a model.*

6. In a session with Carlos, the counselor emphasizes planned contact with feared stimuli, memories or situations while reducing avoidance so fear can be relearned. This BEST illustrates:

- A. Resistance
- B. Exposure therapy**
- C. Transference
- D. Countertransference

Answer: B - Exposure therapy. Exposure reduces avoidance and allows corrective learning; it may be in vivo, imaginal or other formats.

Memory aid: Approach, do not avoid.

7. A therapist working with Nadine uses an intervention characterized by direct, real-life exposure to a feared object or situation. Which approach or technique is being used?

- A. In vivo exposure**
- B. Empty-chair technique
- C. Cognitive restructuring
- D. Countertransference

Answer: A - In vivo exposure. In vivo means exposure in the actual situation rather than only in imagination.

Memory aid: VIVO = in real life.

8. In a session with Zachary, the counselor emphasizes gradual exposure to a fear hierarchy combined with relaxation or another incompatible response. This BEST illustrates:

- A. Systematic desensitization**
- B. Social modeling
- C. Extinction
- D. Exposure therapy

Answer: A - Systematic desensitization. Systematic desensitization classically combines relaxation training with stepwise movement through a fear hierarchy.

Memory aid: RHE: Relax, Hierarchy, Expose gradually.

9. A therapist working with Kemi uses an intervention characterized by rapid and prolonged exposure to a high-intensity feared stimulus or situation without gradual hierarchy progression. Which approach or technique is being used?

- A. Flooding**
- B. Rational Emotive Behavior Therapy (REBT)
- C. Rational emotive imagery
- D. Psychoanalytic therapy

Answer: A - Flooding. Flooding differs from graded exposure because the client confronts a high level of fear more rapidly.

Memory aid: Flooding = jump into the deep end.

10. A therapist working with Hassan uses an intervention characterized by systematically tensing and releasing muscle groups to learn physical relaxation and reduce arousal. Which approach or technique is being used?

- A. Retroflexion
- B. Negative reinforcement
- C. Progressive muscle relaxation**
- D. Resistance

Answer: C - Progressive muscle relaxation. PMR builds awareness of tension and relaxation by working through muscle groups.

Memory aid: Tense - notice - release - breathe.

11. A therapist working with Nadine uses an intervention characterized by a skills-based therapy especially associated with emotion dysregulation and borderline personality disorder, using mindfulness, distress tolerance, emotion regulation and interpersonal effectiveness. Which approach or technique is being used?

- A. Gestalt dream work

B. Operant conditioning

C. Existential therapy

D. Dialectical Behavior Therapy (DBT)

Answer: D - Dialectical Behavior Therapy (DBT). DBT combines acceptance and change strategies and teaches concrete skills for severe emotion dysregulation.

Memory aid: *M-D-E-I: Mindfulness, Distress tolerance, Emotion regulation, Interpersonal effectiveness.*

12. A therapist working with Lucas uses an intervention characterized by behavioral teaching and rehearsal of interpersonal skills such as greetings, eye contact, listening and conversation. Which approach or technique is being used?

A. Social modeling

B. Gestalt therapy

C. Rational Emotive Behavior Therapy (REBT)

D. Social skills training

Answer: D - Social skills training. Social skills training uses instruction, modeling, rehearsal and feedback to build interpersonal behavior.

Memory aid: *IMRF: Instruction, Modeling, Rehearsal, Feedback.*

13. In a session with Joel, the counselor emphasizes the client says whatever thoughts, feelings, images or memories come to mind without censoring them. This BEST illustrates:

A. Operant conditioning

B. Adlerian therapy

C. Free association

D. Retroflexion

Answer: C - Free association. Free association is a classic psychoanalytic technique intended to reduce censorship and reveal unconscious material.

Memory aid: *Psychoanalytic FIRT: Free association, Interpretation, Resistance, Transference.*

14. Which psychotherapy concept is most accurately described by the following: the therapist explains or offers meaning about patterns, defenses, dreams, resistance or behavior?

A. Interpretation

B. Gestalt dream work

C. Retroflexion

D. Existential therapy

Answer: A - Interpretation. Interpretation is the analyst's attempt to make latent or previously unconscious meaning more understandable to the client.

Memory aid: *Interpretation = therapist offers possible meaning.*

15. In a session with Zachary, the counselor emphasizes the client unconsciously avoids material or processes that threaten to bring unacceptable conflict into awareness. This BEST illustrates:

A. Resistance

B. Existential therapy

C. Rational Emotive Behavior Therapy (REBT)

D. Cognitive behavioral therapy (CBT)

Answer: A - Resistance. Resistance is the defensive blocking of threatening material or therapeutic work.

Memory aid: *Resistance = psychological brakes.*

SECTION B: TRUE/FALSE - EXPLANATIONS

1. FALSE. Negative reinforcement weakens behavior.

All reinforcement strengthens behavior; negative reinforcement does so by removing an aversive condition.

2. TRUE. Extinction involves withholding the reinforcement that has maintained a behavior.

When reinforcement stops, the behavior may gradually decrease.

3. TRUE. Systematic desensitization classically combines relaxation with graded exposure.

The client learns relaxation and moves through a fear hierarchy.

4. FALSE. Flooding is always more gradual than systematic desensitization.

Flooding is deliberately high-intensity and comparatively rapid.

5. TRUE. Gestalt therapy emphasizes present-moment awareness.

Here-and-now awareness is central to Gestalt work.

SECTION C: MODEL SHORT ANSWERS

1. Describe the empty-chair technique. (2mks)

- Place an empty chair as if a significant person or part of self were present.
- Client speaks directly to that chair and may switch roles, exploring unfinished feelings in the here-and-now.

Memory aid: *Empty chair makes the absent dialogue present.*

2. Describe progressive muscle relaxation. (2mks)

- Systematically tense a muscle group briefly while noticing tension.
- Release and notice relaxation, then proceed through major muscle groups with calm breathing.

Memory aid: *Tense - notice - release.*

3. Define free association. (1mks)

- Client verbalizes whatever comes to mind without censorship, however trivial or uncomfortable, to help reveal patterns and unconscious material.

Memory aid: *Free means uncensored.*

PART THREE: ETHICAL/LEGAL - ANSWERS

1. During supervision, Nadine describes respecting the client's right to make informed choices about treatment. Which concept should guide the discussion?

- A. Autonomy**
- B. Veracity
- C. Boundary violation
- D. Nonmaleficence

Answer: A - Autonomy. Autonomy supports informed choice, consent and respect for the client's self-determination.

Memory aid: A = Allow informed choice.

2. In professional practice, a situation calls for acting to promote the client's welfare and benefit. Which ethical/legal concept is MOST directly involved?

- A. Beneficence**
- B. Termination and referral
- C. Professional competence
- D. Subpoena response

Answer: A - Beneficence. Beneficence asks the professional to actively contribute to the client's well-being.

Memory aid: BENEFIT = beneficence.

3. A counselor asks which principle or professional concept is defined by avoiding or minimizing harm. The BEST answer is:

- A. Nonmaleficence**
- B. Duty to warn/protect
- C. Fidelity
- D. Ethical response to colleague misconduct

Answer: A - Nonmaleficence. Nonmaleficence is the duty to do no harm and to avoid exposing clients to unnecessary risk.

Memory aid: NON-maleficence = NO harm.

4. During supervision, Fidel describes fair and equitable distribution of services, benefits, burdens and opportunities. Which concept should guide the discussion?

- A. Privileged communication
- B. Professional competence
- C. Mandated reporting
- D. Justice**

Answer: D - Justice. Justice concerns fairness and equitable treatment, including access and resource allocation.

Memory aid: Justice = fairness.

5. In professional practice, a situation calls for truthfulness and honest communication with clients. Which ethical/legal concept is MOST directly involved?

- A. Veracity**
- B. Justice
- C. Gift decision-making
- D. Nonmaleficence

Answer: A - Veracity. Veracity requires accurate, truthful information rather than deception.

Memory aid: VERACITY = verifiable truth.

6. A counselor asks which principle or professional concept is defined by keeping commitments, maintaining trust and being faithful to professional responsibilities. The BEST answer is:

- A. Subpoena response
- B. Fidelity**

- C. Malpractice
- D. Boundary violation

Answer: B - Fidelity. Fidelity concerns loyalty, promises and trust within professional relationships.

Memory aid: *Fidelity = faithful to promises.*

7. In professional practice, a situation calls for an ongoing process in which the client receives understandable information about services, risks, benefits, alternatives, limits of confidentiality and choice. Which ethical/legal concept is MOST directly involved?

- A. Ethical response to colleague misconduct
- B. Informed consent**
- C. Record keeping
- D. Termination and referral

Answer: B - Informed consent. Informed consent is not merely a signature; it is an ongoing communication process supporting voluntary, informed participation.

Memory aid: *BRALC: Benefits, Risks, Alternatives, Limits, Choice.*

8. A counselor asks which principle or professional concept is defined by the professional duty to protect information learned in the therapeutic relationship, subject to recognized legal and ethical exceptions. The BEST answer is:

- A. Professional competence
- B. Confidentiality**
- C. Boundary crossing
- D. Autonomy

Answer: B - Confidentiality. Confidentiality protects client information but is not absolute; limits should be explained early.

Memory aid: *Confidentiality = keep private, know the exceptions.*

9. During supervision, Xavier describes the client's broader right to control access to self, body and personal information. Which concept should guide the discussion?

- A. Boundary crossing
- B. Bartering
- C. Privacy**
- D. Sexual relationship with a current client

Answer: C - Privacy. Privacy is the client's right to be left alone or control access, whereas confidentiality is the professional's duty regarding information received.

Memory aid: *Privacy belongs to the person; confidentiality binds the professional.*

10. In professional practice, a situation calls for a legal protection that may prevent compelled disclosure of certain therapy communications in judicial proceedings, depending on jurisdiction. Which ethical/legal concept is MOST directly involved?

- A. Sexual relationship with a current client
- B. Privileged communication**
- C. Termination and referral
- D. Record keeping

Answer: B - Privileged communication. Privilege is a legal evidentiary concept and is distinct from the broader ethical duty of confidentiality.

Memory aid: *Privilege = courtroom/legal shield.*

11. During supervision, Xavier describes taking reasonable protective action when a client presents a serious, credible threat toward an identifiable person, as required by applicable law and ethics. Which concept should guide the discussion?

- A. Privacy
- B. Duty to warn/protect**
- C. Gift decision-making
- D. Boundary crossing

Answer: B - Duty to warn/protect. The course tests duty to warn/protect as an exception to confidentiality when serious danger to others is present. Actual legal requirements vary by jurisdiction.

Memory aid: *TIP: Threat serious, Identifiable target, Protect reasonably.*

12. A counselor asks which principle or professional concept is defined by reporting suspected abuse or other legally reportable situations to designated authorities when law or professional duty requires it. The BEST answer is:

- A. Technology ethics
- B. Mandated reporting**
- C. Record keeping
- D. Veracity

Answer: B - Mandated reporting. Mandated reporting is a recognized limit to confidentiality; therapists should explain this limit during informed consent.

Memory aid: *Report when law makes reporting mandatory.*

13. During supervision, Rose describes professional negligence in which a duty is breached and the breach causes harm. Which concept should guide the discussion?

- A. Fidelity
- B. Malpractice**
- C. Bartering
- D. Technology ethics

Answer: B - Malpractice. Malpractice requires more than an unfavorable outcome: there must be a professional duty, breach, harm and causal link.

Memory aid: *DBHC: Duty, Breach, Harm, Causation.*

14. In professional practice, a situation calls for practicing only within areas supported by appropriate education, training, supervised experience and current skill. Which ethical/legal concept is MOST directly involved?

- A. Professional competence**
- B. Dual or multiple relationship
- C. Privileged communication
- D. Fidelity

Answer: A - Professional competence. Using techniques without adequate training can expose clients to harm and the professional to ethical or legal liability.

Memory aid: *Know your scope; seek supervision or refer.*

15. In professional practice, a situation calls for a situation in which the therapist has another significant role or relationship with the client in addition to therapy. Which ethical/legal concept is MOST directly involved?

- A. Dual or multiple relationship**
- B. Nonmaleficence
- C. Veracity
- D. Record keeping

Answer: A - Dual or multiple relationship. Multiple relationships are not automatically harmful in every context, but should be avoided or carefully managed when they risk exploitation, impaired objectivity or harm.

Memory aid: *Two roles = review power, objectivity, exploitation and harm.*

SECTION B: MODEL SHORT ANSWERS

1. Differentiate confidentiality from privileged communication. (2mks)

- Confidentiality is an ethical/professional duty.
- Privilege is a legal evidentiary protection against compelled disclosure in certain proceedings and depends on jurisdiction.

Memory aid: *Privilege = courtroom shield.*

2. Define malpractice. (1mks)

- Professional negligence in which a duty is breached and the breach causes harm; commonly considered through duty, breach, causation and damages/harm.

Memory aid: DBHC.

3. Give two elements of informed consent. (1mks)

- Examples: nature/purpose of services, risks, benefits, alternatives, fees, therapist qualifications, limits of confidentiality, voluntary choice, right to ask questions.

Memory aid: BRALC.

4. When is termination ethically appropriate? (1mks)

- When goals are met, treatment is no longer beneficial/needed, the therapist cannot provide competent service, or other legitimate reasons exist - with adequate planning to avoid abandonment.

Memory aid: PLAN.

SECTION C: MODEL SHORT ESSAYS

1. State one key ethical requirement for clinical records. (2mks)

- Records should be accurate, timely, relevant and securely stored with access protected.

Memory aid: ACTS.

2. Why are dual relationships ethically risky? (2mks)

- They can impair objectivity or professional judgment.

- They can increase risk of exploitation, coercion, conflicts of interest or harm.

Memory aid: Power + objectivity + exploitation.

3. What is a boundary violation? (1mks)

- A serious harmful or exploitative breach of therapeutic boundaries.

Memory aid: Violation = harm/exploitation.

PART FOUR: CULTURE / GROUP / COUNSELING PROCESS / TREATMENT PLAN - ANSWERS

1. In counseling practice, Yvette encounters a situation involving collaboratively defining client-centered outcomes that are realistic and observable. Which concept BEST applies?

- A. Goal setting**
- B. Assessment
- C. Performing stage
- D. Rapport building

Answer: A - Goal setting. Goals are stronger when the client participates and the goals are specific enough to evaluate.

Memory aid: SMART: *Specific, Measurable, Achievable, Realistic/Relevant, Time-bound.*

2. A trainee observes implementing agreed therapeutic strategies and monitoring the client's response. This is an example of:

- A. Assessment
- B. Intervention stage**
- C. Skills-development group
- D. Professional interpreter use

Answer: B - Intervention stage. Intervention follows assessment and goal definition and should be linked to the case formulation and treatment plan.

Memory aid: Assess -> Goal -> Intervene -> Review.

3. A trainee observes reviewing progress, consolidating coping skills, planning relapse prevention or referral and ending the professional relationship responsibly. This is an example of:

- A. Termination and follow-up**
- B. Paraphrasing
- C. Cultural competence
- D. Cultural imposition

Answer: A - Termination and follow-up. Termination is a therapeutic phase, not simply stopping appointments.

Memory aid: RECAP: *Review, Evaluate, Consolidate, Aftercare, Part.*

4. A trainee observes attending closely to verbal and nonverbal communication and showing the client that the message has been heard and understood. This is an example of:

- A. Assessment
- B. Treatment plan
- C. Active listening**
- D. Self-disclosure

Answer: C - Active listening. Active listening includes presence, minimal encouragers, reflection, clarification and appropriate nonverbal attending.

Memory aid: SOLER: *Squarely face, Open posture, Lean, Eye contact, Relaxed.*

5. A trainee observes identifying and feeding back the emotional meaning expressed by the client. This is an example of:

- A. Case formulation
- B. Cultural imposition
- C. Group role
- D. Reflection of feeling**

Answer: D - Reflection of feeling. Reflection of feeling helps clients recognize and explore emotions rather than only facts.

Memory aid: Name the feeling, not just the facts.

6. Which counseling/group/cultural concept is BEST described by: restating the essence of the client's content in fresh words without changing its meaning?

- A. Rapport building
- B. Storming stage

C. Paraphrasing

D. Norming stage

Answer: C - Paraphrasing. Paraphrasing checks understanding and encourages elaboration.

Memory aid: *Paraphrase = same meaning, new words.*

7. In counseling practice, Tina encounters a situation involving questions that invite elaboration rather than a yes/no answer. Which concept BEST applies?

A. Support group

B. Open-ended questioning

C. Norming stage

D. Forming stage

Answer: B - Open-ended questioning. Open questions support exploration, while excessive questioning can turn therapy into an interrogation.

Memory aid: *WHAT/HOW usually open more than DID/IS.*

8. Which counseling/group/cultural concept is BEST described by: respectfully pointing out discrepancies between a client's words, behaviors, values or nonverbal cues?

A. Therapeutic confrontation

B. Forming stage

C. Cognitive-behavioral group

D. Reflection of feeling

Answer: A - Therapeutic confrontation. Confrontation is not aggression; it is a careful invitation to examine inconsistency.

Memory aid: *Confront discrepancy, not the person.*

9. In counseling practice, Lucas encounters a situation involving the therapist deliberately shares limited personal information when it is likely to benefit the client rather than meet the therapist's own needs. Which concept BEST applies?

A. Goal setting

B. Cultural competence

C. Treatment plan

D. Self-disclosure

Answer: D - Self-disclosure. Self-disclosure should be purposeful, brief and client-centered.

Memory aid: *WAIT: Why am I telling this?*

10. In counseling practice, Victor encounters a situation involving a collaborative written or explicit plan linking identified problems to goals, measurable objectives, interventions and review. Which concept BEST applies?

A. Self-disclosure

B. Therapeutic confrontation

C. Treatment plan

D. Norming stage

Answer: C - Treatment plan. Treatment plans organize care around the client's specific needs and should be revised according to progress.

Memory aid: *PGOIR: Problem, Goal, Objectives, Interventions, Review.*

11. A trainee observes an organized explanation linking the client's presenting problems with relevant history, triggers, maintaining factors, strengths and a rationale for intervention. This is an example of:

A. Ethnocentrism

B. Norming stage

C. Assessment

D. Case formulation

Answer: D - Case formulation. Formulation goes beyond a diagnosis by explaining why this person is struggling in this way at this time and what may help.

Memory aid: 5P option: *Presenting, Predisposing, Precipitating, Perpetuating, Protective.*

12. A trainee observes evaluating whether objectives are being achieved and modifying the treatment plan when progress is absent, circumstances change or goals are met. This is an example of:

- A. Reflection of feeling
- B. Adjourning stage
- C. Intervention stage

D. Progress review

Answer: D - Progress review. Treatment planning is dynamic; review links evidence of progress to continuation, modification or termination.

Memory aid: Review = *measure, discuss, revise.*

13. A trainee observes ongoing development of awareness, cultural knowledge, clinical skills and meaningful encounters with diverse clients. This is an example of:

- A. Professional interpreter use

B. Cultural competence

- C. Intervention stage
- D. Cognitive-behavioral group

Answer: B - Cultural competence. Cultural competence requires reflective awareness plus knowledge, skills and real encounters; it is not achieved by assuming everyone should conform to the therapist's culture.

Memory aid: AKSE: *Awareness, Knowledge, Skills, Encounters.*

14. In counseling practice, Victor encounters a situation involving judging other cultures primarily by the standards of one's own culture and treating one's own as the norm. Which concept BEST applies?

- A. Intervention stage
- B. Ethnocentrism**
- C. Cultural competence
- D. Professional interpreter use

Answer: B - Ethnocentrism. Ethnocentrism places one's own cultural frame at the center and may distort assessment or communication.

Memory aid: ETHNO-centred = *my culture at the centre.*

15. Which counseling/group/cultural concept is BEST described by: pressuring a client to adopt the therapist's or dominant culture's beliefs, values or practices?

- A. Assessment
- B. Cultural imposition**
- C. Skills-development group
- D. Rapport building

Answer: B - Cultural imposition. Cultural imposition occurs when the helper pushes their own values or practices onto the client.

Memory aid: IMPOSITION = *impose my culture.*

SECTION B: MODEL SHORT ESSAYS

1. Differentiate psychoeducational groups from skills-development groups. (4mks)

- Psychoeducational groups emphasize teaching information/understanding about a problem or condition.
- Skills groups emphasize practicing specific behaviors or coping/interpersonal skills with rehearsal and feedback.

Memory aid: Education = *know*; skills = *do*.

2. Define a treatment plan and list four common elements. (4mks)

- A collaborative roadmap tailored to the client linking assessment/formulation to care.

- Common elements: identified problem(s), goals, measurable objectives, interventions/strategies, responsible persons/frequency as relevant, review/progress criteria.

Memory aid: *PGOIR*.

3. Explain SMART goals. (2mks)

- Specific
- Measurable
- Achievable
- Relevant/Realistic
- Time-bound

Memory aid: *SMART*.

SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

Session: Revision Mock Set 06	Duration: 1hr 30mins
Specialty: Psychotherapy	Coefficient: 04
Eliminatory mark: < 14/20	Training reference: VTI-AEDA examination style

SIMULATION OF REAL WORKING SITUATION PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 06

Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.

DETAILED MODEL ANSWERS - SIMULATION OF REAL WORKING SITUATIONS

SECTION I

1. Group support after bereavement - anchor: Support/psychoeducational bereavement group

A counselor is asked to lead a new group for adults who have recently lost spouses. Members differ in culture, age and length of bereavement. Several say they feel isolated because friends avoid talking about grief.

Model response / marking points (25 marks):

1. Clarify purpose and suitability/screening.
2. Explain informed consent, group confidentiality and its limits.
3. Establish agreements on respect, attendance, turn-taking and nonjudgment.
4. Use a forming-stage orientation and invite introductions at a safe pace.
5. Normalize diversity of grief without prescribing a single timeline.
6. Use active listening, empathic reflection and universality.
7. Attend to cultural/religious meanings of mourning.
8. Prevent monopolizing/scapegoating and monitor group roles.
9. Include psychoeducation/coping where appropriate without replacing member support.
10. Close with summary, grounding and next-session expectations.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: FSNPA stages + AKSE culture + group confidentiality.

2. Confidentiality and threat to another person - anchor: Duty to protect/warn scenario

During therapy, Mohamed, 34, describes intense anger toward a named former partner and gives a specific plan to cause serious harm soon. Mohamed has access to the means described and asks the therapist to promise secrecy.

Model response / marking points (25 marks):

1. Take the threat seriously and assess specificity, intent, timing, means, access, past violence, substances and protective factors.
2. Remind the client of previously explained limits of confidentiality.
3. Do not promise secrecy when serious danger is present.
4. Consult supervisor/legal/emergency procedures promptly where possible without delaying necessary protection.
5. Take reasonable protective steps required by applicable law/policy, which may include emergency services and warning/protecting identifiable persons.
6. Disclose the minimum necessary information.
7. Continue to engage the client in de-escalation and voluntary safety steps if possible.

8. Document assessment, consultations, reasoning and actions.
9. Arrange appropriate emergency/psychiatric evaluation.
10. Follow up on safety and continuity of care.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: *TIP + MIN: Threat, Identifiable target, Protect; Minimum necessary disclosure.*

SECTION II

1. Child abuse disclosure in a group - anchor: Safeguarding/mandated reporting

In an adolescent counseling group, a 14-year-old quietly states that a caregiver has been sexually abusing her. Other members fall silent and the young person asks the counselor not to tell anyone.

Model response / marking points (25 marks):

1. Respond calmly, believe/take the disclosure seriously and thank the young person for telling.
2. Do not conduct a detailed forensic interrogation in the group.
3. Protect privacy within the group as much as possible and arrange a private conversation promptly.
4. Assess immediate safety and urgent medical needs.
5. Explain that abuse is a limit to confidentiality and that required safeguarding/reporting steps must be taken.
6. Follow applicable child-protection law/policy and supervisor/agency procedure without unnecessary delay.
7. Do not confront the alleged abuser personally in a way that could increase danger.
8. Document the disclosure accurately using the child's own words where possible.
9. Support the group after the disclosure without revealing unnecessary details.
10. Arrange trauma-informed follow-up and protective supports.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: *SAFE: Support, Assess immediate danger, Follow reporting law, Ensure follow-up.*

2. Teletherapy privacy and informed consent - anchor: Technology ethics and informed consent

A therapist plans video sessions with Peter, who lives in a crowded home and often joins from a shared device. Connection sometimes drops, and the therapist has not discussed what to do in an emergency.

Model response / marking points (25 marks):

1. Discuss privacy at both therapist and client locations.
2. Use an appropriately secure platform/device and explain relevant limits/risks.
3. Verify identity and current location at sessions when clinically necessary for emergency response.
4. Obtain informed consent specific to teletherapy.
5. Agree on what to do if connection fails.
6. Create an emergency/contact plan and know local emergency resources where relevant.
7. Protect records and access credentials.
8. Clarify whether the client can speak privately and use headphones/alternative location if safe.
9. Practice only within competence and applicable jurisdictional rules.
10. Document consent and contingency plans.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the

vignette.

Memory aid: *SPACE: Security, Privacy, Access/identity, Consent, Emergency plan.*